

ENQUIRY HANDLING · REVIEW

SkinFit Wellness

What happens to an enquiry between the advertising and the consultation chair, and what we would look at before proposing anything.

01

What we found

WHAT WE SAW

Enquiries arrive every day. Only a small number become a seated consultation. The front desk calls people back, and **often reaches them at a moment that doesn't suit them.**

WHY IT HAPPENS

A call back asks the patient for a second moment they never agreed to. **They had already chosen one: it was when they messaged.**

WHY IT'S UNSEEN

Your patient records hold the people who booked. An enquiry that never becomes one never enters them, so **nothing in the clinic counts what is lost.**

WHAT WE PROPOSE

An audit of how enquiries have actually been handled, **at no cost**, before any system is proposed. The same order you use with a patient: assessment first, plan second.

02

In Mr. Gino's words

ON THE PUSH

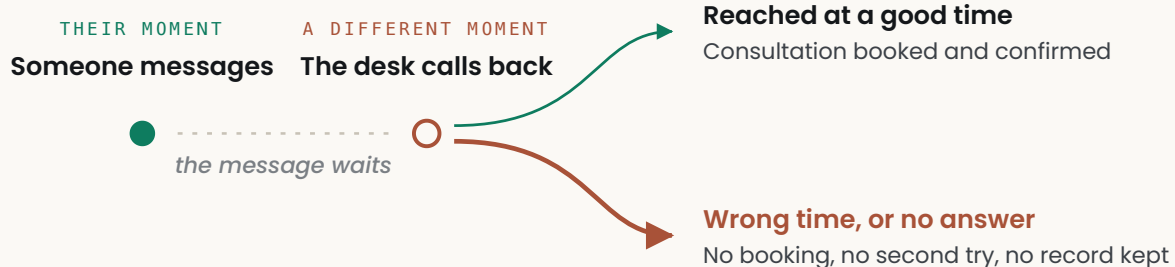
"Since the branch is new we are pushing more towards marketing. But what happens after the marketing, the messages and stuff – that's what I needed to understand."

ON THE DESK

"Right now the front desk is actually picking up phones and calling the ones who enquired, but many times we are catching them at the wrong time."

03

How an enquiry moves today



Your records only ever see the upper path. The lower one leaves no trace, so nothing counts it.

04

Where the gap actually sits

There are two very different standards on either side of the clinic's front door.

Inside, everything follows a set process. A patient is assessed before anything is recommended. The kAI scan measures skin and hair against fixed parameters and produces a written baseline, and the app keeps the patient connected to their doctor between visits. Every step of that was planned in advance.

Outside, there is no set process. A message waits until someone is free. A receptionist calls back in whatever minute she can find between patients. Whether the person is reached, what they actually wanted, and whether anyone tries a second time depends on who is on shift and what they happen to remember.

The clinic has built a careful system for the patient in the chair. The same care has not yet been built for the patient trying to reach you.

05

It gets harder as the marketing works

The more enquiries arrive, the more calls the desk owes, and the fewer free minutes it has to make them from. The advertising also runs around the clock while the desk works a shift: we sent an enquiry through the ₹499 ad on a Saturday evening, and it had not been answered by Monday.

The Koramangala branch is new and spend is rising to fill it. **Whatever share of enquiries becomes a consultation today is the share that will apply to every rupee added after it.** And while the branch is still establishing itself, paid advertising is carrying most of that load for now.

06

What good handling would look like

None of this replaces the front desk. All of it hands your two receptionists a shorter, warmer job than the one they have now.

01

Every enquiry answered immediately, at any hour

NOW	A message waits until someone at the desk is free.
INSTEAD	A real, useful reply within seconds, including at 9:40 on a Saturday night while the ads are still spending.
WHY HERE	Your advertising doesn't keep office hours. Every hour it runs while nobody is at the desk is attention you paid for, arriving where no one can receive it.

02

The concern captured before anyone picks up the phone

NOW	The desk calls back without knowing what the person wants.
INSTEAD	By the time a receptionist is involved, the concern, the treatment being asked about, the branch that suits them and when they are free to talk have all been established.
WHY HERE	This is the direct answer to catching people at the wrong time. You stop guessing the moment, because they have told you.

03

A held appointment instead of a promise to call

NOW	The outcome of an enquiry is a callback attempt.
INSTEAD	The outcome is a consultation booked into the diary and confirmed back to the patient.
WHY HERE	Most people are lost in the gap between “we’ll call you” and someone actually being reached. Removing the step removes the loss.

04

Missed consultations chased the same day

NOW	Whether a no-show is followed up depends on somebody noticing.
INSTEAD	A missed appointment triggers a rebooking offer with specific times, the same day, without anyone having to remember.
WHY HERE	First sessions are priced at ₹499 and ₹1,499 to bring people through the door. That only works if they arrive.

Three more, once those hold

- **Follow-up that continues past the first silence.** These are considered decisions, and almost nothing in this market contacts someone again once they go quiet.
- **Reviews asked for at the right moment.** After a completed visit, from the patients most likely to be glad to leave one.
- **Earlier enquiries revisited.** Worth sizing during the audit rather than promising now, since it depends on consent and platform rules.

07

How we would set this up safely

- 01 **Patient records stay where they are.** We work on incoming messages, not medical records. Your patient system stays yours.
- 02 **Nothing automated ever gives medical advice.** It captures what someone is asking and routes it to your team. It never answers clinical questions or speaks in a doctor's name.
- 03 **We only message people who contacted you first,** within what WhatsApp and Meta permit. No purchased lists, nothing that puts your number at risk.
- 04 **Every message is logged and readable by you.** Nothing goes out unseen.
- 05 **Access and data stay yours.** You grant access and can withdraw it at any time, and we never take a copy of your patients elsewhere.

08

What we'd like to do next

You don't recommend a treatment before the scan. We'd rather not recommend systems before we've seen where the enquiries actually go.

The audit, at no cost to the clinic

We go through how enquiries have been handled over **the past two weeks or more**, across every channel they arrive on: Messenger and the ad forms, Instagram, WhatsApp, the website form, and the calls the front desk makes. The message threads themselves are enough. There is no report to prepare and nothing to export.

Then we walk you through what we found: **which systems we would put in place, in what order, and how the rollout would work.** From there you can judge what is worth doing, and if it makes sense on both sides, we start building.

We would suggest doing this in the coming week. The ads are already live and the reels are still gaining reach. As the targeting settles and more people see the clinic, enquiries will rise rather than fall. A flagship branch with this interior, the experience and the kAI assessment behind it should convert well once someone walks in, but whatever the desk cannot catch at today's volume, it will not catch at a higher one.

It takes about thirty minutes of your team's time. Send us two days that work and we will confirm one.